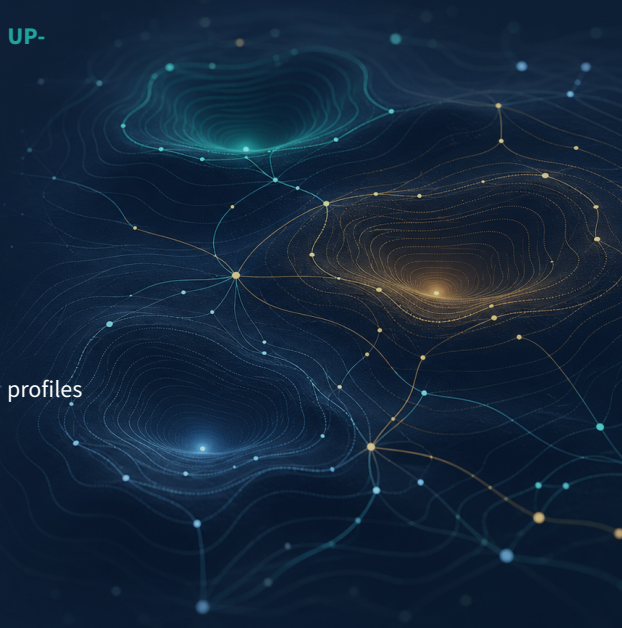
PRELIMINARY RESULTS · SCIENTIFIC ADVISORY UPDATE

FOR2107 First results

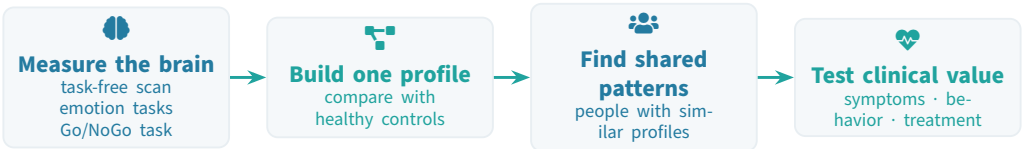
Progress toward reproducing the Tozzi framework

75 scans · 67 selected participants · first circuit profiles

23 July 2026 | What the first evidence shows



The scientific idea



Can brain-circuit patterns reveal clinically meaningful subgroups?

Tozzi et al. answered this using a large sample and several scan conditions. Our current analysis tests the same idea using resting-state scans first.

Where the project stands today

75	67	17	50
RESTING-STATE SCANS	SELECTED PARTICIPANTS	CLINICAL	HEALTHY CONTROLS

Achieved

Resting-state circuit profiles can now be produced for every available participant.

First evidence

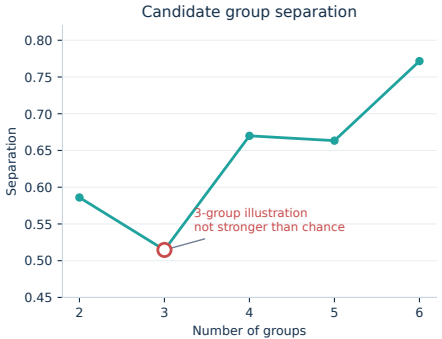
Different average circuit patterns appear when groups are formed.

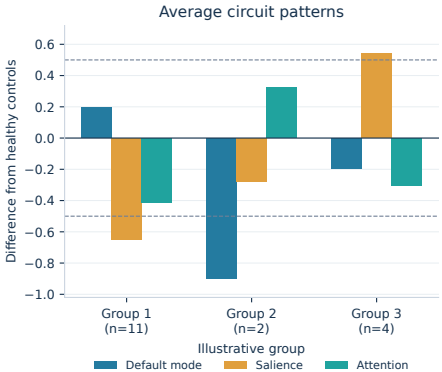
Still needed

Task scans, motion assessment, and a larger clinical sample.

Bottom line: the measurement workflow is ready; the evidence for reproducible patient subgroups is not.

What the first results show





Observable

The available data produce distinct average circuit patterns.

Not established

Separation is not stronger than chance, groups are small, and symptoms do not differ reliably.

How far are we from the published result?

	Tozzi et al.	Current evidence	
Evidence base	801 clinical participants; several studies	17 clinical participants; one current sample	!
Brain information	Rest, emotion, and cognitive control	Resting-state information only	!
Shared principle	Each person summarized by a circuit profile	Same person-level profile concept	✓
Confidence	Repeated and independent confirmation	No independent confirmation yet	!
Conclusion	Six clinically distinct subgroups	**No replicated subgroup yet**	−

The framework matches; the finding is not yet replicated.

What would change the conclusion?

Complete the evidence

Full participant set, task-based measures, and motion assessment.

Challenge the result

Show that the same patterns persist across samples and analysis choices.

Show clinical value

Demonstrate reliable differences in symptoms, behavior, or treatment response.

One design decision remains: without a direct Go/NoGo equivalent, define the goal as either a **partial replication** or a **scientific extension using the closest available task** before analyzing task data.

Current assessment: feasibility demonstrated; replication evidence pending.